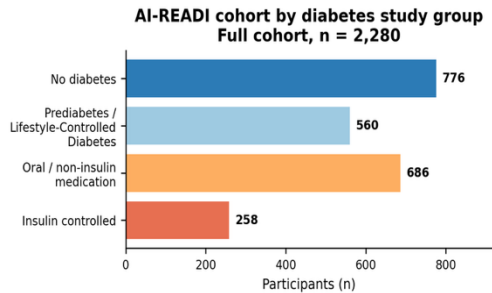


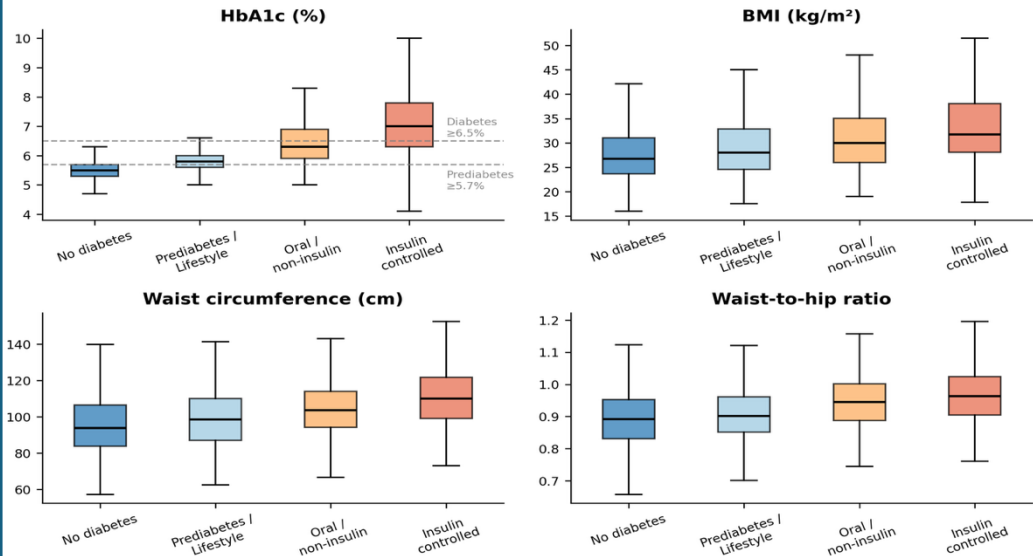
Multimodal Patterns Across Diabetes Severity in AI-READI

1. Who is represented in AI-READI?



2. Do clinical markers follow diabetes severity?

Clinical and body measurements across diabetes groups



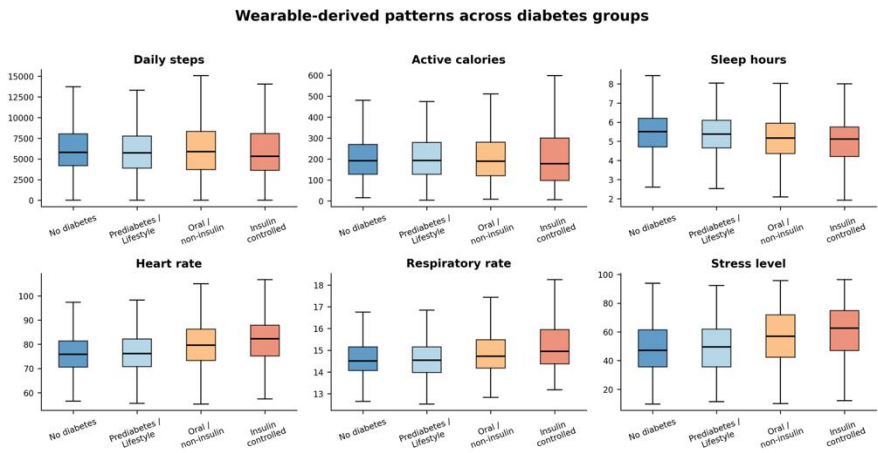
3. What does questionnaire context add?

Self-reported health, behavior, and neighborhood context

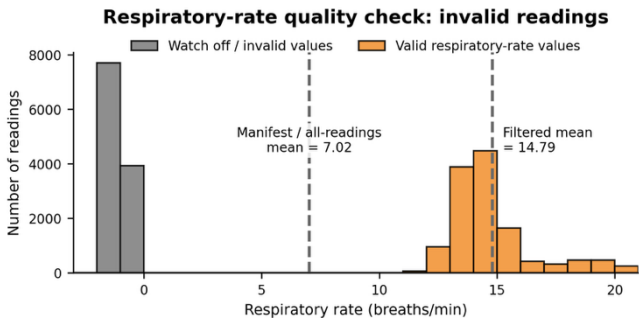
Parent with T2D	0.28	0.43	0.60	0.56	Family history
Sibling with T2D	0.15	0.28	0.41	0.43	
High blood pressure	0.32	0.47	0.66	0.74	Clinical history
High cholesterol	0.37	0.48	0.63	0.62	
Vigorous exercise	6.1	5.8	5.0	4.3	Behavior, food, and mental health
Simple sugar intake	4.0	4.6	4.6	4.8	
Sugary drinks	0.19	0.18	0.24	0.30	
Could not afford balanced meals	0.11	0.14	0.24	0.43	
Depressive symptoms score	8.2	8.8	8.7	9.7	
Ever smoked 100 cigarettes	0.29	0.28	0.33	0.38	
Recreation facilities nearby	3.1	3.0	2.9	2.6	Neighborhood context
Places within walking distance	2.7	2.6	2.5	2.3	
Sidewalks on most streets	3.0	2.9	2.9	2.7	
Shops within walking distance	2.4	2.3	2.3	2.1	
Unsafe walking at night	1.8	1.9	1.9	2.0	

Relative level within feature: Higher (red), Average (white), Lower (blue).

4. Where do wearable patterns overlap, and where do they shift?



5. Why did wearable data need quality checks?



Invalid readings pulled the mean down from 14.79 to 7.02, matching the manifest average.
Example shown for one participant; similar invalid values were common across participants with respiratory-rate data.

Cell values show raw group means. Colors compare groups within each row: blue = relatively lower, white = near average, red = relatively higher.